



HEALTH CARE & CONSENT POLICY

PHN · Prescriber · Dental · Immunization — Minors (MCFD) and Adults (CLBC)

All Residential Care Programs — British Columbia

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Approved By	Executive Director
Review Date	July 1, 2027 (annual); also upon any licensing/funder directive, serious health incident, or BC legislative change
Applies To	All staff at MCFD children/youth programs and CLBC adult programs
Legislative Authority	Infants Act (RSBC 1996, c. 223), s.17 (mature minor consent); Child, Family and Community Service Act (CFCSA); Health Care (Consent) and Care Facility (Admission) Act (RSBC 1996, c. 181) and Health Care Consent Regulation (HCCCFAA); Representation Agreement Act; Patients Property Act (committeeship); Adult Guardianship Act; Public Guardian and Trustee of BC; MCFD SHSS Standards D.6 (rev. April 2026); CLBC service standards
Related Documents	ECC-MED-2026-001 (medication consent, §5); ECC-RC-POL-2026-003; ECC-CSGAC-2026-001; ECC-POL-REG-2026-001

⚠ This policy is a practical operating guide, not legal advice. Consent law is fact-specific. Where any doubt exists about capacity, who may consent, or the scope of a decision-maker's authority, staff STOP and contact the PC/PM, who escalate to the prescriber, the Public Guardian and Trustee (adults), or the MCFD social worker (minors) before proceeding.

Table of Contents

SECTION 1 — Policy Statement	3
SECTION 2 — Definitions	4
SECTION 3 — Consent for Minors (MCFD Stream)	5
3.1 Who Consents	5
3.2 Primary Care / Prescriber (PHN & GP/NP)	5
3.3 Dental Care	6
3.4 Immunization	6
3.5 Documentation — Minors	6
SECTION 4 — Consent for Adults (CLBC Stream)	7
4.1 The Presumption of Capability	7
4.2 If the Adult Is Found Incapable — the Order of Authority	7
4.3 The TSDM Hierarchy	8
4.4 Care Facility Admission Decisions	9
4.5 ECC Intake Practice — Adults	9
SECTION 5 — Primary Care, Dental & Immunization (Adults)	10
5.1 Documentation — Adults	10
SECTION 6 — Documentation, Privacy & Retention	11
SECTION 7 — When Staff Are Unsure	12
APPENDIX A — Intake Consent & Health Information Record (Minors)	12
APPENDIX B — Intake Consent & Support Network Record (Adults)	13
APPENDIX C — Consent Event Log (Both Streams)	14
Version History & Approval	14

SECTION 1 — POLICY STATEMENT

Every person in ECC's care has the right to informed, lawful decision-making about their own health care — whoever makes the decision. This policy governs how staff support access to primary care (PHN/prescriber), dental care, and immunization, and how consent is obtained and documented, across ECC's two funder streams:

MCFD stream (children & youth)	Governed primarily by the Infants Act (mature minor consent) and consent from a parent, guardian, or MCFD as the legal custodian under the CFCSA. See Section 3.
CLBC stream (adults)	Governed by the presumption of adult capacity and the Health Care (Consent) and Care Facility (Admission) Act (HCCCFAA), including representation agreements, committee ship, and temporary substitute decision makers (TSDMs). See Section 4.

Medication-specific consent procedures (prescriptions, OTC authorization, PRN protocols) are governed by ECC-MED-2026-001 §5 and are not repeated in full here — this policy covers the broader health care and consent framework those medication procedures sit inside.

SECTION 2 — DEFINITIONS

Capacity / capable	The ability to understand information relevant to a health care decision and appreciate the reasonably foreseeable consequences. Assessed decision-by-decision, not globally — a person can be capable of some decisions and not others
Mature minor	Under the Infants Act, a person under 19 who a health care provider determines understands the nature and consequences of the proposed care and has considered it in the minor's own interest, and is therefore treated as capable of consenting
Substitute decision maker (SDM)	A general term for anyone lawfully authorized to make a health care decision on behalf of a person who cannot consent for themselves — for adults, this includes a representative, a committee, or a TSDM (Section 4)
Representative	A person named by an adult, while capable, in a Representation Agreement (ss.7 or 9) to make health and/or personal care decisions if the adult later becomes incapable
Committee	A guardian appointed by the BC Supreme Court under the Patients Property Act with authority over an adult's person and/or estate — the highest level of substitute authority
Temporary Substitute Decision Maker (TSDM)	A person selected by the health care provider, following a fixed legal hierarchy (Section 4.3), to give or refuse substitute consent when an incapable adult has no representative or committee
Informed consent	Consent is legally valid only where it relates to the specific care proposed, is given voluntarily, is not obtained by fraud or misrepresentation, and the person was given the information a reasonable person would need — the diagnosis, nature of the proposed care, material risks and benefits, alternatives, and consequences of refusing
Public Guardian and Trustee (PGT)	BC's statutory office of last resort for adults with no available, willing, qualified representative, committee, or TSDM. Reached at 1-877-511-4111

SECTION 3 — CONSENT FOR MINORS (MCFD STREAM)

3.1 Who Consents

Default rule	A parent or guardian with legal custody consents to health care for a child or youth. Where MCFD holds a Continuing Custody Order or the youth is otherwise in MCFD's care, the social worker consents or has delegated consent authority per the placement documentation
Mature minor (Infants Act s.17)	A youth under 19 may consent to their own health care — without parent, guardian, or MCFD consent — if the attending health care provider (physician, dentist, nurse practitioner, etc.) is satisfied the youth understands the nature and consequences of the treatment and has considered it in their own interests. This is the provider's clinical determination, not a staff decision
ECC practice for youth approaching maturity	Even where the Infants Act may apply, ECC staff involve the PC, who consults the MCFD social worker and the prescriber on any consent question involving a youth nearing or asserting mature-minor status — this protects the youth's legal right to independent consent while keeping MCFD informed as legal custodian
Ongoing vs. new care	Existing, previously consented prescriptions do not require re-consent at every administration unless there is a material change (new medication, new diagnosis, significant dose change). A new prescription or new treatment requires fresh informed consent from the appropriate consent-giver before it begins
Emergency care	In a true emergency, care proceeds without waiting for consent; the PC notifies the MCFD social worker immediately after 911/emergency department involvement (see ECC-MED-2026-001 §5, §37)

3.2 Primary Care / Prescriber (PHN & GP/NP)

Personal Health Number (PHN)	Every youth has an active BC PHN on file in their Individual Medication/Health Profile before or immediately at admission; the PC applies for one through Health Insurance BC if the youth does not have one
Attaching to a primary care provider	The PC works to attach every youth to a family physician or nurse practitioner (through Health Connect Registry or a walk-in/community clinic where no attachment is available) within 30 days of admission
New prescription	Prescriber obtains informed consent from the appropriate consent-giver (parent/guardian, MCFD social worker, or the youth if a mature minor) before the treatment begins; PC documents the prescription, the consent-giver, and the consent basis in the youth's health/medication record before the first dose (ECC-MED-2026-001 §5)
Routine appointments	PC schedules and (where appropriate) attends routine and follow-up appointments; prepares a summary of current health status, medications, and concerns for every appointment (ECC-MED-2026-001 §32.2)
Specialist referrals	PC tracks referrals and wait times; keeps the social worker informed of any significant diagnosis or referral outcome

3.3 Dental Care

Coverage	PC confirms dental coverage (MCFD-funded dental program / Healthy Kids or equivalent) at admission; obtains a dental provider if the youth does not have one
Routine care	A dental check-up and cleaning is scheduled within 90 days of admission if the youth's dental history is unknown or care is overdue, then at the routine interval recommended by the dental provider (typically every 6 months)
Consent	Same consent rules as Section 3.1 apply — parent/guardian/MCFD consent, or the youth directly where the dentist determines mature-minor status for the proposed procedure
Urgent dental issues	Pain, injury, swelling, or suspected infection are treated as urgent: PC arranges same-day or next-available emergency dental care; social worker notified

3.4 Immunization

Records	PC obtains the youth's immunization record (from the previous caregiver, school, or Public Health) at admission and enters it in the health profile; gaps are identified
Routine schedule	PC arranges catch-up immunization per the BC immunization schedule through the local public health unit or the youth's primary care provider
Consent	Immunization requires informed consent like any other health care: parent/guardian/MCFD consent, or the mature minor's own consent where the immunizing provider determines the youth understands the nature, risks, and benefits

	and is deciding in their own interest. A youth's informed refusal of immunization as a mature minor is respected and documented; it is not overridden by staff or by MCFD absent a legal basis to do so
School/outbreak requirements	Where a school or public health order requires proof of immunization or exclusion during an outbreak, PC coordinates with public health and the social worker on next steps

3.5 Documentation — Minors

Consent record	Every new prescription, dental procedure, or immunization event records: what was proposed, who consented (name/role), the date, and the basis (parent/guardian, MCFD delegated authority, or mature-minor self-consent) — filed in the Individual Medication/Health Profile
Mature-minor determinations	Where a health care provider determines a youth is a mature minor for a specific decision, the PC notes the provider's name, the decision covered, and the date — this determination is decision-specific and is not assumed to extend to unrelated future decisions
Retention	Health and consent records are retained per Section 6

SECTION 4 — CONSENT FOR ADULTS (CLBC STREAM)

4.1 The Presumption of Capability

Every adult is presumed capable	Under the HCCCFAA, all adults (19+) are presumed capable of giving, refusing, or revoking consent to health care unless a health care provider determines otherwise. Staff never assume incapacity from a diagnosis, communication difference, or disability alone
Support to decide	The adult is asked directly; a representative, family member, or close friend may support (not replace) the adult in understanding and communicating a decision, if the adult wants that support
Capacity is decision-specific and can change	An adult may be capable of consenting to some health care and not other, more complex care; capacity is reassessed if a health care provider has reasonable grounds to believe it may have changed, at any time, even after a decision has already been acted on

4.2 If the Adult Is Found Incapable — the Order of Authority

Where a physician, nurse practitioner, dentist, or other covered health care provider determines an adult is incapable of consenting to specific health care, the provider must obtain substitute consent from the first of the following that exists and applies to that decision:

1	Representative named in a Representation Agreement (s.7 or s.9) — if the agreement covers the health care in question
2	Committee of Person (or Committee of Person and Estate) appointed by the BC Supreme Court under the Patients Property Act
3	A valid Advance Directive — if it directly addresses the specific health care decision, no substitute decision-maker consent is needed; the directive is followed directly
4	Temporary Substitute Decision Maker (TSDM) — chosen by the health care provider from the statutory hierarchy in Section 4.3, where none of the above exist or apply
5	Public Guardian and Trustee (PGT) — as a last resort, either authorizing a willing person to act as TSDM or acting as TSDM directly, where no one from the hierarchy is available, willing, and qualified, or an unresolvable dispute exists (1-877-511-4111)

4.3 The TSDM Hierarchy (used only when 1–3 above do not apply)

The health care provider — not ECC staff, and not the adult — must select the first available, willing, and qualified person from this fixed statutory order. A person qualifies only if they are 19 or older, capable, have had contact with the adult in the past 12 months, and have no dispute with the adult:

1	Spouse (married, common-law, or same-sex; length of cohabitation does not matter)
2	Adult child (all equally ranked)
3	Parent (equally ranked)
4	Sibling (equally ranked)
5	Grandparent (equally ranked)
6	Grandchild (equally ranked)
7	Anyone else related by birth or adoption
8	A close friend (a long-term, close personal relationship with frequent contact, who is not paid to provide the adult's care)
9	A person related by marriage only (in-laws, step-parents, step-children)
ECC's role	ECC staff do not select the TSDM — that is the health care provider's legal duty. ECC's role is to: know who is in the adult's support network (recorded at admission, Section 4.5), provide that information to the health care provider when asked, and support the chosen TSDM's ability to be consulted (calls returned, visits facilitated, timely information shared)
Duty to disclose information	Staff and the PC must disclose information a TSDM needs to make an informed decision — including from the service record — even information that would otherwise be confidential, because the Act requires it for this purpose
Authority is time-limited	A TSDM's authority is specific to the health care decision described in their Notice of Authorization; the health care provider must reconfirm authority no more than 21 days before treatment covered by an ongoing decision begins, and must reassess capability if there is reason to believe the adult has become capable
Limits on a TSDM	A TSDM cannot consent to: sterilization for non-therapeutic reasons; certain research; and cannot refuse consent to life-preserving care unless there is substantial agreement among the treating providers that refusal is medically appropriate.

	Involuntary psychiatric treatment under the Mental Health Act is not governed by this Act at all
Disputes	If two equally-ranked people disagree and the health care provider cannot resolve it, or no one on the list is available/willing/qualified, the provider contacts the PGT (1-877-511-4111)

4.4 Care Facility Admission Decisions

Different from health care consent	Admission to, or continued residence in, a care facility is assessed and consented to separately from health care treatment decisions, under the same Act but its own assessment process
Assessor's duty	An independent assessor determines whether the adult is capable of deciding about admission; the adult must be told the purpose of the assessment and that a substitute may decide on their behalf if found incapable
ECC's role	The PC/PM supports the assessment process (providing relevant information to the assessor) but does not conduct the capability assessment — that is the assessor's exclusive role

4.5 ECC Intake Practice — Adults

At admission	PC records: any existing Representation Agreement, Advance Directive, or Committee order (obtain copies); the adult's known support network in TSDM-hierarchy order (spouse, adult children, parents, siblings, etc. — names and contact information) in case a TSDM is ever needed; the adult's own expressed wishes and values about health care, in their own words
No assumptions	Staff never treat a family member as an automatic decision-maker without confirming their legal status (representative, committee, or correctly-selected TSDM) for the specific decision at hand
Ordinary daily care	Day-to-day support (meals, routine hygiene assistance, medication administration exactly as prescribed) is not a 'health care decision' requiring this framework — this framework applies to consenting to proposed medical, dental, and similar treatment

SECTION 5 — PRIMARY CARE, DENTAL & IMMUNIZATION (ADULTS)

PHN & attachment	Every adult has an active PHN and is attached to a family physician or nurse practitioner where possible; PC assists with Health Connect Registry registration if unattached
New treatment	Informed consent obtained per Section 4 before any new treatment, medication change, or procedure begins; documented per Section 4.6
Routine health monitoring	PC coordinates annual health checks and any condition-specific monitoring identified in the individual service plan
Dental care	PC arranges a dental provider and routine care (typically every 6 months, or per the dentist's recommendation); consent per Section 4; urgent dental issues (pain, injury, swelling) arranged same-day or next available
Immunization	PC arranges routine and recommended immunizations (including annual influenza and any public-health-recommended vaccines) through the adult's primary care provider or public health; consent per Section 4; an adult's informed refusal is respected and documented
Advance care planning	PC supports the adult, while capable, to consider an Advance Directive or Representation Agreement if they wish one — this is the adult's choice; ECC does not draft legal documents but can connect the adult with a notary, lawyer, or the Nidus Personal Planning Resource Centre

5.1 Documentation — Adults

Consent record	Every new treatment, dental procedure, or immunization event records: what was proposed, who consented (the adult, or the representative/committee/TSDM by name and legal basis), the date, and — where a TSDM was used — a copy or note of the health care provider's written confirmation of the TSDM decision
TSDM notices	Any Notice of Authorization or written TSDM decision provided by a health care provider is filed in the adult's health record
Retention	Records retained per Section 6

SECTION 6 — DOCUMENTATION, PRIVACY & RETENTION

Where recorded	Health care and consent records are kept in each person's Individual Medication/Health Profile and service record, separate from general daily logs where practical, and clearly labelled
Confidentiality	Health information is shared only with those who have a professional need to know (health care providers, the PC/PM, MCFD social worker or CLBC facilitator, and — for adults — a validly authorized representative/committee/TSDM). Staff do not discuss a person's health information with family or others who lack a legal basis to receive it, except as this policy specifically directs (e.g., TSDM disclosure duty, Section 4.3)
Retention	Health and consent records are retained for the duration of the person's time in care plus a minimum of 7 years (10 years where the record involves controlled substances), consistent with ECC-MED-2026-001 §6.1 and Appendix G, and are never destroyed while care continues
Records belong to the operator	Service and health records are maintained by ECC and are not removed from the program; copies are provided to the funder, the person, or their lawful decision-maker on request per applicable privacy law

SECTION 7 — WHEN STAFF ARE UNSURE

General rule	If there is any doubt about who may consent, whether someone qualifies as a decision-maker, or whether a decision falls within a decision-maker's authority: STOP, do not proceed, and contact the PC
Minors (MCFD)	PC contacts the MCFD social worker; if the question concerns mature-minor status, the PC also confirms with the treating health care provider, whose clinical determination governs
Adults (CLBC)	PC contacts the CLBC facilitator; for a decision-maker dispute or hierarchy question, the treating health care provider contacts the Public Guardian and Trustee (1-877-511-4111) — ECC does not resolve decision-maker disputes itself
Emergency	Emergency care is never delayed to resolve a consent question — see ECC-MED-2026-001 §5 (Emergency medication) and §37
Escalation to PM/ED	Any situation involving a suspected abusive or exploitative substitute decision-maker, a serious disagreement affecting care, or a legal dispute is raised with the PM immediately and the ED same day

APPENDIX A — INTAKE CONSENT & HEALTH INFORMATION RECORD (MINORS)

Youth legal name / DOB	
PHN	
Legal custody status / consent authority (parent, guardian, MCFD — CCO/TCO/agreement)	
MCFD social worker name / contact	
Family physician / nurse practitioner	
Dental provider	
Immunization record obtained? (Y/N, source, date)	
Known allergies / significant health history	
Mature-minor consent noted for any current treatment? (provider, decision, date)	
Completed by (PC) / Date	

APPENDIX B — INTAKE CONSENT & SUPPORT NETWORK RECORD (ADULTS)

Adult's legal name / DOB	
PHN	
Representation Agreement on file? (Y/N — copy attached)	
Advance Directive on file? (Y/N — copy attached)	
Committee order on file? (Y/N — copy attached)	
CLBC facilitator name / contact	
Support network in TSDM order (name, relationship, contact) — spouse / adult child / parent / sibling / grandparent / grandchild / other relative / close friend	
Adult's own expressed wishes about health care (in their words)	
Family physician / nurse practitioner	
Dental provider	
Completed by (PC) / Date	

APPENDIX C — CONSENT EVENT LOG (BOTH STREAMS)

One row per new treatment, dental procedure, or immunization event.

Date	Treatment / Procedure Proposed	Consent Given By	Legal Basis (parent/MCFD/mature minor; adult/rep/committee/TSDM)	Documented By / Filed

Version History & Approval

Version 1.0 — July 2026	Initial policy: minor consent under the Infants Act and CFCSA; adult capability presumption and the full HCCCFAA substitute-decision hierarchy (representative, committee, advance directive, TSDM, PGT); care facility admission assessment; PHN/prescriber, dental, and immunization procedures for both streams; documentation, privacy, and retention; escalation pathway. Appendices A–C.
Executive Director — Signature / Date	